

Started on	Sunday, 12 February 2023, 8:20 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:20 PM
Time taken	12 secs
Marks	0/56
Grade	0 out of 100



Question 1

Not answered

Marked out of 1

Which of the following is true of Huntington disease?

Select one:

- ☐ Genetic defect located on the long arm of chromosome 4
- ☐ Autosomal recessive inheritance
- ☐ Higher than expected suicide rate
- ☐ Onset usually in early adulthood (20s)
- ☐ Results from an expanded CGG repeat

Your answer is incorrect.

Explanation:

Huntington disease is a rare neurodegenerative disorder of the central nervous system, with a genetic autosomal dominant inheritance. It results from the expansion of a CAG trinucleotide repeat in the HTT (huntingtin) gene on the short arm of chromosome 4. Onset is usually during the 30s and 40s. There is a small number of juvenile-onset cases. An increased risk of suicide in Huntington disease is well documented, with rates significantly higher than those of the general population as well as other neurodegenerative diseases.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 166.

Kachian ZR et al. Suicidal ideation and behavior in Huntington's disease: Systematic review and recommendations. Journal of affective disorders. 2019 May 1;250:319-29.

The correct answer is: Higher than expected suicide rate

Question 2

Not answered

Marked out of 1

In which of the following dementias is recognition memory better than recall memory?

Select one:

- ☐ Alzheimer dementia
- ☐ Prion disease dementia
- ☐ Lewy body dementia
- ☐ HIV-associated dementia
- ☐ Frontotemporal dementia (Pick type)

Your answer is incorrect.

Explanation:

HIV-associated dementia is a subcortical dementia. Recall memory is more impaired than recognition memory.



Characteristic	Subcortical dementia	Cortical dementia
Language	No aphasia (anomia, if severe)	Aphasia early
Memory	Impaired recall>recognition	Recall and recognition impaired
Attention and immediate recall	Impaired	Impaired
Visuospatial skills	Impaired	Impaired
Calculation	Preserved until late	Involved early
Executive function	Disproportionately affected	Impairment consistent with other involvement
Speed of cognitive processing	Slowed early	Normal until late in disease
Personality	Apathetic, inert	Unconcerned
Mood	Depressed	Euthymic
Speech	Dysarthric	Articulate until late
Posture	Bowed or extended	Upright
Coordination	Impaired	Normal until late
Motor speed and control	Slowed	Normal
Adventitious movements	Chorea, tremor, tics, dystonia	Absent (Alzheimer dementia –some myoclonus)
Abstraction	Impaired	Impaired

Adapted from Kaplan and Sadock, 12th Ed. 2022. p. 244.



Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 149.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 244.

The correct answer is: HIV-associated dementia

Question 3

Not answered

Marked out of 1

A 42-year-old man has been experiencing troublesome motor activity for the last three years. He also has symptoms of irritability, anxiety, and low mood. He turns up an hour late for his medical appointment and shouts at the receptionist for not being punctual. What is the most likely diagnosis?

Select one:

- ☐ Creutzfeldt-Jakob disease
- ☐ Tourette syndrome
- ☐ Chronic motor tic disorder
- ☐ Huntington disease
- ☐ Wilson disease

Your answer is incorrect.

Explanation:

Huntington disease is a rare neurodegenerative disorder of the central nervous system, with a genetic autosomal dominant inheritance. Onset is usually during the 30s and 40s. Worsening chorea is characterised by initial jerks, tics, gross involuntary movements of all parts of the body, grimacing, and dysarthria. Anxiety and depression are common, as is executive dysfunction with impulsivity and aggression.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

Sampe D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 166.

The correct answer is: Huntington disease

Question **4**

Not answered

Marked out of 1

The greatest risk factor for vascular dementia is

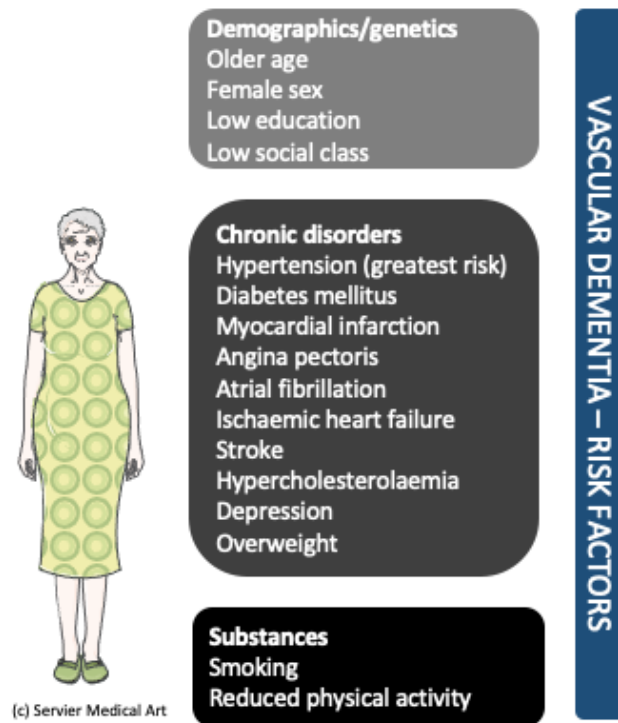
Select one:

- ☐ Hyperlipidaemia
- ☐ Ischaemic heart disease
- ☐ Hypertension
- ☐ Smoking
- ☐ Alcohol consumption

Your answer is incorrect.

Explanation:

The greatest risk factors for stroke and white matter hyperintensities and, consequently, for vascular dementia are high systolic and high diastolic blood pressure.



Adapted from van der Flier et al. 2018



Ref: Van Der Flier WM et al. Vascular cognitive impairment. Nature Reviews Disease Primers. 2018 Feb 15;4(1):1-6.

The correct answer is: Hypertension

Question 5

Not answered

Marked out of 1

Hyperhomocysteinaemia is a modifiable risk factor for

Select one:

- ☐ Bipolar disorder
- ☐ Schizophrenia
- ☐ Dementia
- ☐ Autism
- ☐ Depression

Your answer is incorrect.

Explanation:

Elevated plasma total homocysteine is a modifiable risk factor for the development of cognitive decline, dementia, and Alzheimer disease in older persons.

Ref: Smith AD et al. Homocysteine and dementia: an international consensus statement. Journal of Alzheimer's Disease. 2018 Jan 1;62(2):561-70.

The correct answer is: Dementia



Question 6

Not answered

Marked out of 1

Which of the following is true for late onset schizophrenia?

Select one:

- ☐ Response to treatment is much better than early onset schizophrenia
- ☐ A strong family history is often noted
- ☐ There is usually a life long history of poor social and occupational functioning
- ☐ Sensory impairments are seldom associated
- ☐ It is seen more often in females than males

Your answer is incorrect.

Late-onset schizophrenia is seen more often in females than males. Gender distribution is estimated as 4 to 9 female predominance per male. Sensory impairments such as deafness is commonly associated. Compared to young onset schizophrenia, there is less often a strong family history in late onset schizophrenia.

The correct answer is: It is seen more often in females than males

Question 7

Not answered

Marked out of 1

A 77-year-old man has developed Parkinson's disease. He starts sending his wife sexually abusive text messages and offensive comments, which is new and out of character. Which of the following medications is most likely to have caused this?

Select one:

- ☐ Clonazepam
- ☐ Ropinirole
- ☐ Alpha methyl dopa
- ☐ L-dopa
- ☐ Buspirone

Your answer is incorrect.

L-dopa administration in Parkinson's disease can lead to increased levels of brain dopamine that may play a role in the development of hypomania and manic symptoms in such patients.

The correct answer is: L-dopa



Question 8

Not answered

Marked out of 1

What is the most common psychological and behavioural symptom in patients with Huntington disease?

Select one:

- ☐ Reduction in drive
- ☐ Agitation
- ☐ Aggression
- ☐ Disinhibition
- ☐ Poor self care

Your answer is incorrect.

Explanation:

Apathy is 'a quantifiable reduction in goal-directed behaviour', manifesting clinically as a reduction in interest, spontaneity, motivation, and drive. It is the most frequent psychological and behavioural symptom in Huntington disease (HD), especially in the middle and later stages, causing a severe reduction in activities of daily living. With regard to cognitive and psychological symptoms, apathy and irritability are the two faces of the same coin. A patient can be apathetic in the morning and irritable in the afternoon, depending on the situation. As is the case for irritability, apathy can be caused by environmental and psychological issues. Apathy may also be an adaptive response when the patient feels overwhelmed by too much stimulation (HD patients are more sensitive to noise and environmental interferences), or with the feeling that their disease is progressing.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

The correct answer is: Reduction in drive

Question 9

Not answered

Marked out of 1

Deficits on tests of attention and visuospatial ability may be especially prominent in

Select one:

- ☐ Frontotemporal dementia
- ☐ Vascular dementia
- ☐ Lewy body dementia
- ☐ Alzheimer disease
- ☐ Creutzfeldt-Jakob disease

Your answer is incorrect.

Explanation:

The pattern of cognitive impairment in dementia with Lewy bodies (DLB) differs from that seen in Alzheimer disease in that short-term memory loss is less prominent (as seen predominantly in Alzheimer disease), but deficits in visuospatial, attention, and executive cognitive domains are more pronounced. Deficits on tests of attention, executive function, and visuospatial ability may be especially prominent and occur early in DLB.

Ref: Sanford AM. Lewy body dementia. Clinics in geriatric medicine. 2018 Nov 1;34(4):603-15.

McKeith IG et al. Diagnosis and management of dementia with Lewy bodies: Fourth consensus report of the DLB Consortium. Neurology. 2017 Jul 4;89(1):88-100.

The correct answer is: Lewy body dementia

Question 10

Not answered

Marked out of 1

A 63-year-old woman with a mixed affective state also suffers from psoriasis. Which drug is most likely to worsen her skin condition?

Select one:

- ☐ Lamotrigine
- ☐ Olanzapine
- ☐ Quetiapine
- ☐ Sodium valproate
- ☐ Lithium

Your answer is incorrect.

Explanation:

Some skin conditions such as psoriasis and acne can be aggravated by lithium therapy.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 250.

The correct answer is: Lithium



Question 11

Not answered

Marked out of 1

A 74-year-old man presents with a history of forgetfulness and some confusion at times. Which of the following factors is protective against the development of dementia?

Select one:

- ☐ Reduced levels of physical activity
- ☐ Higher education
- ☐ History of depression
- ☐ Cardiovascular illness
- ☐ Smoking

Your answer is incorrect.

Explanation:

There is an association between lower educational levels and a higher incidence of dementia and Alzheimer disease. The cognitive reserve hypothesis suggests that education and intellectual stimulation extend neural reserve. Therefore, the brain can withstand more insults before developing clinical symptoms of dementia.

Risk and protective factors	Impact on risk of dementia and Alzheimer disease (AD)
Age and gender	Largest risk factor for dementia and AD is advancing age Women have higher prevalence and incidence of dementia than men
Education and intellectual stimulation	Lower educational levels associated with higher incidence of dementia and AD Cognitive reserve hypothesis: education and intellectual stimulation extend neural reserve; brain can withstand more insults before developing dementia
Psychosocial factors	Increased social engagement associated with lower rates of cognitive decline Depression is possible risk factor for developing dementia
Genetic susceptibility	First-degree relatives of people with AD have higher risk of developing AD Young-onset AD: main genetic cause is mutations in three genes - amyloid precursor protein (APP), presenilin 1, and presenilin 2 Late-onset AD: complex interplay between genetic predisposition and environmental risk factors; apolipoprotein E (APOE) E4 allele confers susceptibility while APOE E2 allele is protective
Vascular risk factors	Smoking, high cholesterol, high blood pressure, and diabetes mellitus are all associated with increased risk of AD
Hearing loss	Increased risk of dementia in people with hearing loss

Adapted from Lilford and Hughes 2020

SPMM COURSE

Ref: Lilford P, Hughes JC. Epidemiology and mental illness in old age. BJPsych Advances. 2020 Mar;26(2):92-103.

The correct answer is: Higher education

Question 12

Not answered

Marked out of 1

Which of the following interventions is not recommended when managing delirium in the elderly?

Select one:

- ☐ Reduce unnecessary noise
- ☐ Achieve optimal levels of environmental stimulation
- ☐ Preferably use low dose haloperidol for patients who require drug treatment
- ☐ Ensure the safety of all patients and their immediate surroundings
- ☐ Preferably use benzodiazepines for patients who require drug treatment

Your answer is incorrect.

Explanation:

When managing delirium, it should be ensured that the environment is safe. Optimise stimulation by ensuring adequate lighting, reducing unnecessary noise, mobilising the patient when possible, and correcting any sensory impairment (e.g. hearing aids, glasses). Psychotropic medication should be used with caution in delirium and only for situations that have not yielded to non-pharmacological interventions. Conventionally, the antipsychotic of choice is haloperidol, given its absence of anticholinergic side effects. The use of antipsychotics is controversial, and there is little evidence to indicate that they should be used to treat delirium directly. There is no evidence to support the use of benzodiazepines in managing delirium not associated with alcohol or benzodiazepine withdrawal.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 855.

FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30:1-11.

The correct answer is: Preferably use benzodiazepines for patients who require drug treatment

Question 13

Not answered

Marked out of 1

Which one of the following statements about neurotic disorders in the elderly is not true?

Select one:

- ☐ Non specific anxiety symptoms, hypochondriacal and depressive symptoms predominate
- ☐ Multiple factors may contribute to new neurotic symptoms in the elderly
- ☐ The estimated prevalence of neurotic disorders is between 1-10%
- ☐ Males are predominantly affected than females
- ☐ Obsessional, dissociative and conversion disorders are less common

Your answer is incorrect.

The estimated prevalence of neurotic disorders is between 1-10% with a female predominance. Non-specific anxiety symptoms, Hypochondriacal and depressive symptoms predominate. Obsessional, phobic, Dissociative and conversion disorders are less common. There is an increase in referrals to psychiatry due to decreased acceptance of symptoms by the elderly. Multiple factors may contribute to new neurotic symptoms in the elderly. The most common ones are; physical illness, major life events, bereavement, social isolation, impaired self-care and insecure personality factors.

The correct answer is: Males are predominantly affected than females



Question **14**

Not answered

Marked out of 1

Which of the following neuroimaging techniques that may be used when assessing dementia is helpful in studying white matter lesions and vascular lesions?

Select one:

- ☐ Skull x-ray
- ☐ CT scan
- ☐ PET scan
- ☐ MRI scan
- ☐ SPECT scan

Your answer is incorrect.

Explanation:

MRI scans are useful in examining for neoplasms, vascular malformations, seizure foci, demyelinating disorders, neurodegenerative disorders, and infarctions. MRI better discriminates the interface between grey and white matter and is useful in detecting a variety of white matter lesions in the periventricular and subcortical regions. White matter abnormalities are detected in older patients with hypertension, vascular dementia, and dementia of the Alzheimer type.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 226.

The correct answer is: MRI scan

Question 15

Not answered

Marked out of 1

A 70-year-old man has been diagnosed with Lewy body dementia. He is being started on rivastigmine. Which of the following side effects is most likely to occur?

Select one:

- ☐ Falls
- ☐ Nausea
- ☐ Blurred vision
- ☐ Prostatism
- ☐ Syncope

Your answer is incorrect.

Explanation:

The most common side effects of rivastigmine are anorexia, dizziness, nausea, vomiting, and diarrhoea.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 605.

The correct answer is: Nausea

Question 16

Not answered

Marked out of 1

The most common psychiatric side effect of levo-dopa among the following is

Select one:

- ☐ Anxiety attacks
- ☐ Delirium
- ☐ Nightmares
- ☐ Delusional syndrome
- ☐ Night terrors

Your answer is incorrect.

Up to 1/3rd of patients complain of psychiatric side effects when taking levodopa therapy. Nearly 30% of those taking levodopa complain of nightmares and vivid dreams. This is followed by 7% with night terrors, 5% with delirium and 3% with delusional disorder.

The correct answer is: Nightmares



Question 17

Not answered

Marked out of 1

The neurotransmitters most often implicated in the pathophysiology of Alzheimer disease are

Select one:

- ☐ Acetylcholine and dopamine
- ☐ Acetylcholine and serotonin
- ☐ Dopamine and norepinephrine
- ☐ Dopamine and serotonin
- ☐ Acetylcholine and norepinephrine

Your answer is incorrect.

Explanation:

The neurotransmitters usually implicated in the pathophysiologic condition of Alzheimer disease (AD) are acetylcholine and norepinephrine, both of which are likely hypoactive in AD. Under normal circumstances, neurons in the basal nucleus of Meynert synthesise acetylcholine (ACh) and send axonal projections throughout the entire cerebral cortex and the limbic system to provide cholinergic innervation. Loss of neurons in the basal nucleus of Meynert characterises AD. As with the distribution of the macroscopic and microscopic changes in AD, ACh activity is particularly reduced in the cortical association areas and limbic system. AD is also associated with reduced concentrations of other established or putative neurotransmitters: somatostatin, substance P, norepinephrine (noradrenaline), vasopressin, and several other polypeptides. However, compared to the ACh loss, their concentrations are not decreased profoundly or consistently.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 240.

Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 122.

The correct answer is: Acetylcholine and norepinephrine

Question 18

Not answered

Marked out of 1

The personality disorder that is most prevalent in older adults is

Select one:

- ☐ Borderline
- ☐ Paranoid
- ☐ Obsessive-compulsive
- ☐ Dependent
- ☐ Schizoid

Your answer is incorrect.

Explanation:

Personality disorder diagnoses usually seen in late life include the dependent, obsessive-compulsive, histrionic, paranoid, schizoid, and avoidant types. Studies suggest that these diagnoses remain stable over the lifespan or even increase. In several studies, obsessive-compulsive personality disorder has been found to be the most prevalent in older adults. Some studies indicate that borderline personality disorders are less prevalent among older people than among younger people.

Ref: Bangash A. Personality disorders in later life: epidemiology, presentation and management. BJPsych Advances. 2020 Jul;26(4):208-18.

Penders KA et al. Personality disorders in older adults: A review of epidemiology, assessment, and treatment. Current Psychiatry Reports. 2020 Mar;22(3):1-4.

The correct answer is: Obsessive-compulsive

Question 19

Not answered

Marked out of 1

Which of the following is contraindicated in the elderly with psychosis and cognitive impairment?

Select one:

- ☐ Haloperidol
- ☐ Promazine
- ☐ Risperidone
- ☐ Quetiapine
- ☐ Sulpride

Your answer is incorrect.

Having reviewed the literature on the use of Olanzapine and Risperidone for the behavioural and psychological symptoms of dementia, the committee on safety of medicines concluded that each was associated with at least two-fold increase in the risk of stroke and should, therefore, be no longer used in dementia (Duff 2004)

The correct answer is: Risperidone

Question 20

Not answered

Marked out of 1

Older people with dementia often do not tolerate drugs as well as a healthy older person. Which of the following is contraindicated in patients with dementia?

Select one:

- ☐ Sulpiride
- ☐ Haloperidol
- ☐ Promazine
- ☐ Quetiapine
- ☐ Olanzapine

Your answer is incorrect.

The chairman of the CSM concluded that risperidone and olanzapine are associated with at least a two-fold increase in the risk of stroke and, therefore, should no longer be used in dementia (Duff, 2004). Herrmann et al. (2004), however, found no difference in the risk of stroke between risperidone and olanzapine compared with typical neuroleptics used in the treatment of behavioural problems in those with dementia (n=11400). Others have subsequently commented on the potential detrimental effects of such a blanket ban (Mowat et al, 2004) (Karim and Byrne, Treatment of psychosis in elderly people | BJPsych Advances. (2005). Retrieved from <http://apt.rcpsych.org/content/11/4/286>). Promazine has a relatively good side effect profile and so is suitable for the elderly. It is not effective for schizophrenia (Maudsley Guidelines. 2007) Excerpted from Karim and Byrne. Treatment of psychosis in elderly people Advances in Psychiatric Treatment (2005) 11: 286-296

The correct answer is: Olanzapine

Question **21**

Not answered

Marked out of 1

Compared to other subtypes, paranoid subtype of schizophrenia is more often associated with which of the following features?

Select one:

- ☐ Older age at onset
- ☐ Poorer functional recovery
- ☐ More cognitive deficits
- ☐ Poorer premorbid abilities
- ☐ Poorer occupational functioning

Your answer is incorrect.

Features such as poorer premorbid function, poorer psychosocial recovery, worse cognitive deficits and earlier age of onset are associated with hebephrenic or disorganized schizophrenia.

The correct answer is: Older age at onset



Question **22**

Not answered

Marked out of 1

Factors associated with a better prognosis in Alzheimer disease include

Select one:

- ☐ Medial temporal lobe atrophy
- ☐ Severe executive dysfunction
- ☐ Family history of dementia
- ☐ Onset after age 65 years
- ☐ Male sex

Your answer is incorrect.

Explanation:

Data suggest that in persons with an early onset of dementia or with a family history of dementia, the disease is likely to have a rapid course. Male sex and older age have been associated with increased risk of mortality in Alzheimer dementia (AD). Other predisposing factors for an increased risk of mortality are more severe executive dysfunction, presence of medial temporal lobe atrophy, and higher p-tau in CSF, indicative of more severe AD pathology.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 238.

Rhodijs-Meester HF et al. Disease-related determinants are associated with mortality in dementia due to Alzheimer's disease. Alzheimer's research & therapy. 2018 Dec;10(1):1-10.

The correct answer is: Onset after age 65 years

Question **23**

Not answered

Marked out of 1

Which of the following is a good prognostic factor in late life depression?

Select one:

- ☐ Co-morbid physical illness
- ☐ Severe life events during follow up period
- ☐ Severity of initial illness
- ☐ Psychotic symptoms
- ☐ Onset less than age 70 years

Your answer is incorrect.

Explanation:



Late life depression	Good prognostic factors
	• <70 years old at onset • Short duration of illness • Absent physical illness • Good previous adjustment • Good previous recovery
	Poor prognostic factors
	• Poor medication adherence • Severity of initial illness • Comorbid physical illness • Presence of psychotic symptoms • Severe life events following the onset

Ref: Oxford Handbook of Psychiatry , 4th Ed, 2019. p. 553.

SPMM COURSE



Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 553.

The correct answer is: Onset less than age 70 years



Question **24**

Not answered

Marked out of 1

The response to antidepressants is expected to be generally poor in

Select one:

- ☐ Patients with vascular depression
- ☐ Patients with late onset depression
- ☐ Patients with early onset depression
- ☐ Patients with psychotic depression
- ☐ Patients with loss of weight

Your answer is incorrect.

Alexopoulos et al. (1997) proposed that cerebral ischaemic damage to the frontal-subcortical circuits could predispose, precipitate and perpetuate late-onset depression. Simpson et al. studied the association between subcortical disease and antidepressant response in late-life depression, concluding that a poor response could be expected in patients with vascular depression.

The correct answer is: Patients with vascular depression

Question 25

Not answered

Marked out of 1

A 76-year-old woman presents to her GP upon her daughter's insistence. She exhibits features of very-late-onset schizophrenia-like psychosis. Which of the following types of delusions is most likely to be seen?

Select one:

- ☐ Hypochondriacal delusions
- ☐ Partition delusions
- ☐ Delusions of reference
- ☐ Religious delusions
- ☐ Delusions of misidentification

Your answer is incorrect.

Explanation:

Critical differences exist in the clinical presentation of LOS and VLOS compared with the early-onset type. Researchers have generally found a female preponderance, lower genetic risk (reflected by a lower incidence of family history of psychosis), and higher rates of and more severe paranoid symptoms and persecutory delusions in LOS and VLOS patients. Partition delusions – concerning the unwanted permeability of borders – are highly prominent.

Ref: Suen YN et al. Late-onset psychosis and very-late-onset-schizophrenia-like-psychosis: an updated systematic review. International Review of Psychiatry. 2019 Aug 18;31(5-6):523-42.

Van Assche L et al. The neuropsychology and neurobiology of late-onset schizophrenia and very-late-onset schizophrenia-like psychosis: a critical review. Neuroscience & Biobehavioral Reviews. 2017 Dec 1;83:604-21.

The correct answer is: Partition delusions

Question **26**

Not answered

Marked out of 1

The most prevalent anxiety disorder among older adults is

Select one:

- ☐ Social anxiety disorder
- ☐ Agoraphobia
- ☐ Generalised anxiety disorder
- ☐ Specific phobia
- ☐ Panic disorder

Your answer is incorrect.

Explanation:

The estimated prevalence of anxiety disorders in older adults is 4-14%. They are more common among women than men. Specific phobia is the most prevalent anxiety disorder among older adults; fear of falling is most common. Generalised anxiety disorder is one of the most common anxiety disorders in older adults; anxiety content reflects multiple issues that arise in later stages of life (e.g. health, welfare or loss of loved ones, life transitions, retirement, caregiving responsibilities), and economic and legal issues (e.g. reduced income, increased health care costs, end-of-life planning). Onset of social anxiety disorder is typically not in late life. Panic disorder is relatively infrequent in older adults and onset is typically in younger adulthood.

Ref: Ramos K, Stanley MA. Anxiety disorders in late life. *Psychiatric Clinics*. 2018 Mar 1;41(1):55-64.

The correct answer is: Specific phobia

Question **27**

Not answered

Marked out of 1

A 65-year-old man admitted to a neurology unit has an unsteady gait and repeated falls. This is most likely to be associated with

Select one:

- ☐ Neuroleptic malignant syndrome
- ☐ Alzheimer dementia
- ☐ Delirium
- ☐ Parkinson disease
- ☐ Polypharmacy

Your answer is incorrect.

Explanation:

Polypharmacy has been repeatedly linked to fall incidence and gait disturbances in older adults.

Ref: Montero-Odasso M et al. Polypharmacy, Gait Performance, and Falls in Community-Dwelling Older Adults. Results from the Gait and Brain Study. Journal of the American Geriatrics Society. 2019 Jun;67(6):1182-8.

The correct answer is: Polypharmacy



Question **28**

Not answered

Marked out of 1

According to NINCDS-ADRDA criteria, which of the following is NOT a criterion required to diagnose probable Alzheimer's disease?

Select one:

- ☐ Absence of systemic diseases or other brain diseases that could explain the cognitive changes
- ☐ Cognitive deficit is seen in more than one domain
- ☐ Onset between ages 40 and 90
- ☐ Presence of cerebral atrophy in neuroimaging studies
- ☐ No disturbance in level of consciousness

Your answer is incorrect.

Cognitive deficits must be seen in 2 or more domains. The absence of loss of consciousness, presence of cerebral atrophy, an appropriate age of onset and absence of other systemic diseases to explain cognitive deficits support the diagnosis of probable AD.

The correct answer is: Presence of cerebral atrophy in neuroimaging studies



Question 29

Not answered

Marked out of 1

Compared with early-onset depression, older adults with late-onset depression tend to present more often with symptoms of

Select one:

- ☐ Worthlessness
- ☐ Psychomotor change
- ☐ Hopelessness
- ☐ Suicidal thoughts
- ☐ Anhedonia

Your answer is incorrect.

Explanation:

There are no clear distinctions between the clinical presentation of depression in the elderly and that in younger patients. However, some symptoms are often more striking. Severe psychomotor retardation or agitation occurs in up to 30% of depressed elderly patients. Depressive delusions regarding poverty, physical illness, or nihilistic in nature are common. Classic symptoms may not be evident, and the patient may instead present with somatic, anxiety, or hypochondriacal complaints. A high index of suspicion is required when older patients present with these symptoms, especially abnormal illness behaviour.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 552.

The correct answer is: Psychomotor change

Question **30**

Not answered

Marked out of 1

A 75-year-old man with Parkinson disease develops psychosis unrelated to levodopa use. Of those listed, which is the best antipsychotic for his treatment?

Select one:

- ☐ Risperidone
- ☐ Haloperidol
- ☐ Quetiapine
- ☐ Aripiprazole
- ☐ Olanzapine

Your answer is incorrect.

Explanation:

Clozapine, quetiapine, and pimavanserin have shown some efficacy in treating psychosis in Parkinson disease. Clozapine has the most robust evidence base. However, its use is limited by agranulocytosis risk and the associated need for blood count monitoring. In clinical practice, quetiapine is more readily used than clozapine, given its side effect profile compared to clozapine. Other neuroleptics have thus far demonstrated mixed results with increased risk of extrapyramidal worsening. The introduction of pimavanserin has provided another treatment option for Parkinson disease psychosis, decreasing concern for deterioration in motor function.

Ref: Kyle K, Bronstein JM. Treatment of psychosis in Parkinson's disease and dementia with Lewy Bodies: A review. *Parkinsonism & Related Disorders*. 2020 Jun 1;75:55-62.

The correct answer is: Quetiapine

Question **31**

Not answered

Marked out of 1

Which of the following best indicates the status of estrogen treatment in postmenopausal women with Alzheimer's disease?

Select one:

- ☐ RCTs find no improvement in the Mini Mental State Examination (MMSE) scores
- ☐ No RCTs have been conducted to examine the effects of estrogen in this clinical group
- ☐ RCTs consistently show beneficial effects on activities of daily living
- ☐ RCTs find an improvement in MMSE scores that is sustained for at least one year
- ☐ RCTs find small short-term improvements in MMSE scores that do not translate to clinical benefit

Your answer is incorrect.

A Cochrane systematic review identified five RCTs that included 210 postmenopausal women with Alzheimer disease. "The reviewers looked at various outcomes (such as MMSE, memory, concentration, and cognitive function) after treatment with different types of estrogen, at different doses, delivered by different routes and for different durations. Meta-analyses showed that there was a limited positive effect from low-dose conjugated equine estrogens (0.625 mg once a day) but not from a higher dose (1.25 mg of conjugated equine estrogens once a day) on the MMSE after two months, but the effect disappeared after three, six, and 12 months of treatment. This effect was small and not clinically relevant". (<http://journals.plos.org/plosmedicine/article?id=10.1371/journal.pmed.0020031>)

The correct answer is: RCTs find small short-term improvements in MMSE scores that do not translate to clinical benefit

Question **32**

Not answered

Marked out of 1

Which of the following is not a neurobiological feature of the normal ageing process?

Select one:

- ☐ Loss of nerve cells in the hippocampus
- ☐ Reduction of the ventricular size
- ☐ Reduction in the weight of the brain
- ☐ Reduction in cerebral blood flow in the frontal lobe
- ☐ Reduced cortical grey matter

Your answer is incorrect.

Explanation:

Biological changes in the brain associated with ageing include:

- Decrease in gross brain weight, about 17% by age 80 in both sexes
- Widened sulci, smaller convolutions, gyral atrophy
- Ventricles enlarge
- Increased transport across blood-brain barrier
- Decreased cerebral blood flow and oxygenation

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 915.

The correct answer is: Reduction of the ventricular size

Question 33

Not answered

Marked out of 1

Which of the following statements concerning pharmacokinetics in the elderly is true?

Select one:

- ☐ Renal blood flow decreases with age even in those with normal physical health
- ☐ Total gastrointestinal drug absorption is reduced
- ☐ Gastric pH decreases with age
- ☐ 25% of adult renal function is lost by 80 years
- ☐ Serum creatinine measurement is the most accurate way of measuring renal function

Your answer is incorrect.

Explanation:

How ageing affects drug therapy (pharmacokinetics):

- Gut motility decreases with age, as does gastric acid secretion; drugs are absorbed more slowly but the same amount is absorbed
- Renal function declines with age; 35% of function is lost by 65 years and 50% by age 80
- Measurement of serum creatinine can be misleading because muscle mass is reduced, so less creatinine is produced

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 602.

The correct answer is: Renal blood flow decreases with age even in those with normal physical health

Question **34**

Not answered

Marked out of 1

A 70 year old lady has been diagnosed with Alzheimer's dementia recently. What is the risk of same illness for her sister provided there is no other family history of Alzheimer's dementia?

Select one:

- ☐ Risk will be reduced
- ☐ Risk will be three times higher
- ☐ Risk will be 10 times higher
- ☐ Same risk as general population
- ☐ Risk will double

Your answer is incorrect.

The actual predicted risk of developing Alzheimer's disease in the first-degree relatives of probands with Alzheimer's disease is 15-19%, compared with 5% in controls. Thus, the risk to the first-degree relatives of patients with Alzheimer's disease who developed the disorder at any time up to the age of 85 years is increased some 3 - 4 times relative to the risk in controls. This would seem to translate to a risk of developing Alzheimer's disease of between one in five and one in six. Liddell et al. (2001) Genetic risk of Alzheimer's disease: advising relatives. The British Journal of Psychiatry, 178, 7-11.

The correct answer is: Risk will be three times higher

Question **35**

Not answered

Marked out of 1

You are seeing a 77-year-old man with late onset psychosis. He has a history of falls due to postural hypotension. Which drug is most likely to cause orthostatic hypotension?

Select one:

- ☐ Lorazepam
- ☐ Amisulpride
- ☐ Olanzapine
- ☐ Risperidone
- ☐ Haloperidol

Your answer is incorrect.

Risperidone and Clozapine are two drugs, which have a greater risk of causing severe postural hypotension. The suggested drugs are amisulpride, aripiprazole, haloperidol and sulpiride.

The correct answer is: Risperidone



Question **36**

Not answered

Marked out of 1

Which of the following antidementia drugs produces significant improvements in delusions and hallucinations seen in patients with Lewy Body dementia?

Select one:

- ☐ Tacrine hydrochloride
- ☐ Donepezil
- ☐ Galantamine
- ☐ Rivastigmine
- ☐ Memantine

Your answer is incorrect.

A large multicenter, double-blind trial comparing Rivastigmine with placebo in patients with LBD showed significant improvements in delusions and hallucinations (Mc Keith et al. 2000). Cholinesterase inhibitors are not yet licensed for the treatment of LBD in UK but substantial provisional evidence suggest that AChEIs are effective in some LBD cases for improvement in cognitive functions and also in treating neuropsychiatric features such as apathy, agitation and psychosis.

The correct answer is: Rivastigmine



Question **37**

Not answered

Marked out of 1

A 70-year-old gentleman suffers from Parkinson's disease and is also currently depressed. His wife says he has been sending her sexually explicit texts and also accused her of having an affair. This is a new type of behaviour and is out of keeping with his previous pattern of behaviour when depressed. Which drug is most likely to account for this presentation?

Select one:

- ☐ Diazepam
- ☐ Duloxetine
- ☐ Propranolol
- ☐ Ropinirole
- ☐ Dothiepin

Your answer is incorrect.

Ropinirole can be used by itself or in conjunction with levodopa to treat Parkinson's disease, and is particularly useful in people for whom levodopa alone is no longer providing adequate control of symptom. Ropinirole works by stimulating these same receptor sites. This produces the same effects as dopamine and acts as a dopamine substitute. Increased sex drive (libido) and excessive interest or involvement in sexual activity (hypersexuality) is uncommon but possible side effect of this medication (Retrieved from <http://www.netdoctor.co.uk/seniors-health/medicines/requip.html>).

The correct answer is: Ropinirole

Question **38**

Not answered

Marked out of 1

An elderly lady reports smelling of gas passed through the walls and thought her neighbours are trying to poison her. She has contacted the police on a number of occasions over last 3 months claiming that neighbours are spying on her. The most likely diagnosis is;

Select one:

- ☐ Delusional disorder
- ☐ Diogenes syndrome
- ☐ Schizophrenia
- ☐ Alzheimer's disease
- ☐ Delirium

Your answer is incorrect.

Late-onset schizophrenia is characterized by paranoid delusions and hallucinations. The patients usually complain of partition delusions as described above and may harbour persecutory delusions against neighbours. Social isolation and sensory deprivation are significant contributory factors for psychosis in the elderly. Confusingly, ICD-10 includes paraphrenia along with paranoid schizophrenia, and also includes the term paraphrenia with delusional disorders. In the presence of hallucinations, schizophrenia is the most appropriate diagnosis.

The correct answer is: Schizophrenia



Question 39

Not answered

Marked out of 1

Which of the following is not a recognized neuroimaging finding in late-onset depression?

Select one:

- ☐ Reduction in the volume of the caudate nucleus
- ☐ Evidence of vascular disease
- ☐ Reduced hippocampal volume
- ☐ Shrinkage of the ventricles
- ☐ Reduction in grey matter volume in the frontal and temporal lobes

Your answer is incorrect.

Explanation:

Depression, including late-onset depression, has been repeatedly associated with ventricular enlargement (not shrinkage of the ventricles).

Ref: Zhuo C et al. The rise and fall of MRI studies in major depressive disorder. Translational psychiatry. 2019 Dec 9;9(1):1-4.

The correct answer is: Shrinkage of the ventricles

Question 40

Not answered

Marked out of 1

Which one of the following statements about learning and memory in the elderly is not true?

Select one:

- ☐ The elderly take longer than young persons to learn new material
- ☐ Simple recall remains intact
- ☐ Memory encoding ability diminishes
- ☐ IQ remains stable until approximately 70 years of age
- ☐ Complete learning of new materials still occurs

Your answer is incorrect.

Explanation:

Simple recall has been repeatedly shown to decline with age.

Ref: Graves LV, Moreno CC, Seewald M, Holden HM, Van Etten EJ, Uttarwar V, McDonald CR, Delano-Wood L, Bondi MW, Woods SP, Delis DC. Effects of age and gender on recall and recognition discriminability. Archives of Clinical Neuropsychology. 2017 Dec 1;32(8):972-9.

The correct answer is: Simple recall remains intact



Question **41**

Not answered

Marked out of 1

Which class of antidepressants is generally used as first-line treatment for late-onset depression?

Select one:

- ☐ SNRIs
- ☐ SSRIs
- ☐ Trazodone
- ☐ MAOI inhibitors
- ☐ Tricyclic antidepressants

Your answer is incorrect.

Explanation:

A Cochrane review examining the efficacy, and associated withdrawal rates, of different classes of antidepressants in older people found that SSRIs and tricyclics are of the same efficacy. However, TCAs are associated with higher withdrawal rates. NICE recommends starting with an SSRI (sertraline is commonly used first line in older people). There is no ideal antidepressant in older people, and all are associated with problems. TCAs are broadly considered less desirable due to the increased risk of cardiac conduction abnormalities and anticholinergic effects. SSRIs are generally better tolerated than TCAs; however, they increase the risk of bleeding. Older people are also particularly prone to develop hyponatraemia with SSRIs, as well as postural hypotension and falls.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 673-676.

The correct answer is: SSRIs

Question 42

Not answered

Marked out of 1

Which one of the following is not a prion disease?

Select one:

- ☐ Creutzfeldt-Jakob Disease
- ☐ Gerstmann-Straussler-Scheinker syndrome
- ☐ Fatal familial insomnia
- ☐ Kuru
- ☐ Steele-Richardson syndrome

Your answer is incorrect.

Explanation:

Prion diseases are rare, rapidly progressive dementing illnesses caused by the spread of deposits of abnormal prion protein (PRNP) throughout the brain as a result of either inherited genetic mutation, sporadic mutation, or infection. The typical pathological finding is spongy encephalopathy. Prion diseases include Creutzfeldt-Jakob disease, new variant CJD, fatal familial insomnia, Gerstmann-Straussler-Scheinker syndrome, and kuru.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 168.

The correct answer is: Steele-Richardson syndrome

Question 43

Not answered

Marked out of 1

Classically, vascular dementia has been distinguished from dementia of the Alzheimer type by

Select one:

- ☐ History of stroke
- ☐ Absence of insight
- ☐ Female predominance
- ☐ Insidious onset
- ☐ Stepwise deterioration

Your answer is incorrect.

Explanation:

Classically, vascular dementia has been distinguished from dementia of Alzheimer type by the decremental (stepwise) deterioration that can accompany cerebrovascular disease over time. Although this discrete, stepwise deterioration may not be apparent in all cases, focal neurological symptoms and standard risk factors for cerebrovascular disease are more common in vascular dementia than dementia of the Alzheimer type.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 236.

The correct answer is: Stepwise deterioration

Question **44**

Not answered

Marked out of 1

The type of dementia seen in patients with Parkinson disease is

Select one:

- ☐ Pseudodementia
- ☐ Both cortical and subcortical
- ☐ Cerebellar
- ☐ Cortical
- ☐ Subcortical

Your answer is incorrect.

Explanation:



Classification of dementias

Cortical	Subcortical	Cortico-subcortical
• Alzheimer disease • Frontotemporal dementia (including "Pick disease")	• Vascular dementia (including Biswanger disease) • Progressive supranuclear palsy • Multiple system atrophy • Huntington disease • Parkinson disease • Wilson disease • Multiple sclerosis • Hydrocephalus • HIV-associated dementia	• Dementia with Lewy bodies • Creutzfeldt-Jacob disease (multi-focal, rapid, and aggressive) • Corticobasal degeneration

Adapted from Shaik and Varma 2012



Ref: Shaik SS, Varma AR. Differentiating the dementias: a neurological approach. Progress in Neurology and Psychiatry. 2012 Jan;16(1):11-8.
The correct answer is: Subcortical

Question **45**

Not answered

Marked out of 1

Features suggestive of pseudodementia include all except

Select one:

- ☐ A past history of depression and/or an identifiable precipitant
- ☐ Onset can be dated with some precision
- ☐ Subjective complaints about poor memory
- ☐ "Don't know" answers typical in cognitive tests
- ☐ Long duration before medical help is sought

Your answer is incorrect.

Explanation:



Features	Depressive-related cognitive dysfunction ('Pseudodementia')	Major Neurocognitive Disorder (Dementia)
Onset	Can be dated with some precision	Dated only within broad terms
Duration before help-seeking	Symptoms usually of short duration before medical help is sought	Symptoms usually of long duration before medical help is sought
Progression	Rapid progression of symptoms after onset	Slow progression of symptoms
Past psychiatric history	Common	Not very common
Subjective memory loss	Often present	Often absent
Patient distress	Usually strong sense of distress	Patients often appear unconcerned
Affective changes	Often pervasive	Affect labile and shallow
Social skills	Loss often early and prominent	Social skills often retained
Nocturnal worsening	Uncommon	Often present
Attention	Often well preserved	Reduced, especially in later stages
Cognitive tests	'Don't know' answers typical	Near-miss answers frequent
Memory loss	Both recent and remote events	Recent events usually more severe

Adapted from Kaplan and Sadock's Synopsis of Psychiatry, 12th ed. 2022. p. 237.

SPMM COURSE



Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 237.

The correct answer is: Long duration before medical help is sought

Question 46

Not answered

Marked out of 1

Which of the following is true with regard to the pharmacokinetics of psychotropics in the elderly?

Select one:

- ☐ The volume of distribution of fat soluble drugs is reduced
- ☐ By the age of 80, nearly 25% of renal function is lost
- ☐ The rate of absorption of drugs is slower
- ☐ Serum creatinine accurately reflects renal function
- ☐ The amount of drug absorbed after oral intake is reduced

Your answer is incorrect.

The rate of absorption is slower but nearly equal absorption of oral administered drugs. Absorption is not greatly affected. GI blood flow is diminished. Gastric pH is increased as acidity drops. Decreases in renal blood flow have been approximated at 10% per decade beginning after the fourth decade - leads to reduced creatinine clearance and GFR. A larger volume of distribution and longer half-life of lipophilic chemicals noted because of their increased sequestration in fat e.g.; Benzodiazepines are excreted more slowly in elderly.

The correct answer is: The rate of absorption of drugs is slower

Question **47**

Not answered

Marked out of 1

Picks disease is characterised by all except

Select one:

- ☐ There is positive family history in 50% of cases
- ☐ It is caused by Autosomal dominant mutation in the Tau gene (chromosome 17) with complete penetration
- ☐ There is prominent atrophy of the white matter with relative sparing of the grey matter
- ☐ Knife blade atrophy on CT/MRI scan is commonly seen in most cases
- ☐ The onset is usually between 45-65 years

Your answer is incorrect.

In Pick's Disease, CT/MRI brain reveals bilateral symmetrical abnormalities of the frontal and temporal lobes. A milder generalised atrophy but a marked atrophy of frontal and temporal lobes with sparing of the posterior third of the superior temporal gyrus produces the so-called Knife blade atrophy. Both grey and white matter is equally affected with blurring of the boundary.

The correct answer is: There is prominent atrophy of the white matter with relative sparing of the grey matter



Question **48**

Not answered

Marked out of 1

Which of the following features is not related to the biological changes seen with normal aging process?

Select one:

- ☐ Thinning of the optic lens
- ☐ Progressive hearing loss particularly at the high frequencies
- ☐ Decrease in melatonin
- ☐ Impaired T-cell response to antigens
- ☐ Decrease in cardiac output and stroke volume

Your answer is incorrect.

Changes in bodily function during old age includes a decrease in cardiac output and stroke volume, a decrease in glomerular filtration rate, a reduction in cerebral blood flow and oxygenation, decreased vital capacity in the lungs, decrease in melatonin, thickening of the optic lens in association with an inability to accommodate and progressive hearing loss particularly at the high frequencies.

The correct answer is: Thinning of the optic lens



Question 49

Not answered

Marked out of 1

Mrs. Y is an 83-year-old lady whose sister has been diagnosed with early Alzheimer's disease. She is worried that she may develop the same condition. What is her risk compared to a person who lacks such a family history?

Select one:

- ☐ Ten times as likely
- ☐ Twice as likely
- ☐ Thrice as likely
- ☐ Same risk
- ☐ Five times as likely

Your answer is incorrect.

An actual predicted risk of developing Alzheimer's disease in the first-degree relatives of probands with Alzheimer's disease is 15-19%, compared with 5% in controls. Thus, the risk to the first-degree relatives of patients with Alzheimer's disease who developed the disorder at any time up to the age of 85 years is increased some 3 - 4 times relative to the risk in controls (Retrieved from <http://bjp.rcpsych.org/content/178/1/7>).

The correct answer is: Thrice as likely

Question **50**

Not answered

Marked out of 1

Which of the following can be used as a test of executive functioning in an older person with schizophrenia?

Select one:

- ☐ Boston naming test
- ☐ Letter number span
- ☐ California verbal learning test
- ☐ Tower of Hanoi
- ☐ Controlled oral word association test

Your answer is incorrect.

Explanation:

A popular test of problem-solving/executive functioning is the Tower of London (or Hanoi) task, which requires subjects to move coloured discs onto three posts to obtain target designs in a set number of moves. It can be used to test frontal lobe function.

Ref: Hodges JR. Cognitive assessment for clinicians, 3rd Edition. 2018. pp. 24, 28-29.

The correct answer is: Tower of Hanoi

Question **51**

Not answered

Marked out of 1

Ginkgo is a popular herbal remedy for memory enhancement. Patients treated for which of the following conditions should avoid ginkgo?

Select one:

- ☐ Treated for bacterial pharyngitis with amoxicillin
- ☐ Treated for dental pain with ibuprofen
- ☐ Treated for atrial fibrillation with warfarin
- ☐ Treated for Paget disease with alendronate
- ☐ Treated for dementia with rivastigmine

Your answer is incorrect.

Explanation:

Several reports have noted that ginkgo may increase the risk of bleeding. The drug is widely used in Germany but less so elsewhere.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 618.

The correct answer is: Treated for atrial fibrillation with warfarin

Question 52

Not answered

Marked out of 1

In dementia with Lewy bodies, visual hallucinations occur in up to

Select one:

- ☐ 80% of cases
- ☐ 20% of cases
- ☐ 10% of cases
- ☐ 50% of cases
- ☐ 40% of cases

Your answer is incorrect.

Explanation:

Recurrent, complex visual hallucinations occur in up to 80% of patients with dementia with Lewy bodies. They are a frequent clinical signpost to diagnosis. The hallucinations are typically well-formed, featuring people, children, or animals, sometimes accompanied by related phenomena including passage hallucinations, sense of presence, and visual hallucinations.

Ref: McKeith IG et al. Diagnosis and management of dementia with Lewy bodies: Fourth consensus report of the DLB Consortium. Neurology. 2017 Jul 4;89(1):88-100.

The correct answer is: 80% of cases

Question 53

Not answered

Marked out of 1

The Hachinski Score is a way of looking into risk factors for

Select one:

- ☐ Frontotemporal dementia
- ☐ Alzheimer dementia
- ☐ Vascular dementia
- ☐ Pick disease
- ☐ Lewy body dementia

Your answer is incorrect.

Explanation:

The modified Hachinski Ischaemic Score (HIS) is used as a measure of the likelihood of vascular pathology based on the nature of dementia presentation and vascular risk factors. The HIS contains 8 items and scores on the HIS range from 0 to 12: a single point is scored for 4 of the items, with two points scored for each of the remaining 4 items. Clinically a score of greater than 4 on the HIS is indicative of a diagnosis of vascular dementia.

Ref: Titheradge D et al. Cambridge Cognitive Examination and Hachinski Ischemic Score as predictors of MRI confirmed pathology in dementia: A cross-sectional study. International Journal Of Clinical Practice. 2020 Feb;74(2):e13446.

The correct answer is: Vascular dementia

Question **54**

Not answered

Marked out of 1

In Parkinson disease-related psychosis, hallucinations in which sensory modality are the most common?

Select one:

- ☐ Auditory
- ☐ Tactile
- ☐ Visual
- ☐ Olfactory
- ☐ Gustatory

Your answer is incorrect.

Explanation:

Psychosis in Parkinson disease (PD) is a spectrum of neuropsychiatric manifestations consisting of 'positive symptoms', namely illusions, hallucinations, and delusions. Psychosis in PD is often characterised by visual hallucinations. Auditory hallucinations and delusions occur far less frequently, and usually in younger patients. Tactile, olfactory, or somatic hallucinations are quite rare.

Ref: Jones S et al. Neuropsychiatric symptoms in Parkinson's disease: aetiology, diagnosis and treatment. BJPsych Advances. 2020:1-10.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 815.

Marinus J et al. Risk factors for non-motor symptoms in Parkinson's disease. The Lancet Neurology. 2018 Jun 1;17(6):559-68.

The correct answer is: Visual

Question **55**

Not answered

Marked out of 1

A characteristic feature in diagnosis of Lewy body dementia is

Select one:

- ☐ Repeated falls
- ☐ Fluctuating level of consciousness
- ☐ Visual hallucinations
- ☐ Parkinsonian features
- ☐ Sensitivity to neuroleptics

Your answer is incorrect.

Visual hallucinations can be present in more than 80% of patients with LBD.

The correct answer is: Visual hallucinations



Question 56

Not answered

Marked out of 1

Which one of the following is classified as a 'behavioural symptom' seen in dementia?

Select one:

- ☐ Aphasia
- ☐ Delusions
- ☐ Short term memory impairment
- ☐ Wandering
- ☐ Auditory hallucinations

Your answer is incorrect.

Explanation:

Behavioural and psychological symptoms of dementia (BPSD) occur in over 90% of patients to varying degrees. Behavioural symptoms include aggression, wandering, explosive temper, sexual disinhibition, inappropriate toileting, excessive eating, and searching behaviour.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 644.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 157.

The correct answer is: Wandering



© 2023 SPMM Course Limited, UK.

[Refund Policy](#) | [Terms & Conditions](#)

